Adjuvant ribociclib + endocrine therapy

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

Make treatment fit into daily life

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

Before starting

Confirm the exact medicine, strength, endocrine partner, baseline tests and first review appointments. Ask who arranges tests and who reviews the results.

Taking treatment

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

Treatment duration

At completion of the planned three-year ribociclib course, continue the separately agreed endocrine and follow-up plan.

Supply and appointments

Request the next supply before tablets run out. Blood tests and any electrocardiogram (ECG) or injections need their own appointments. This oral treatment does not itself require a central line or infusion pump.

If treatment is held

Ask which medicine is held, whether the endocrine partner continues, what needs to recover, when the next review is due and who will authorise restart.

Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible. New severe symptoms need prompt assessment rather than waiting for the next blood test.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00892 v2 · official regimen
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NICE NG101 · bone health and follow-up
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · implanted port patient information
- eviQ · peripherally inserted central catheter (PICC) patient information
- NCCP NCIS antiemetics v8 (2025), Table 1
- NATALEE · 4-year exploratory analysis (2025)
- Novartis · Kisqali product information, updated September 2026
- EMA Kisqali current product information; checked 2026-10-25
- NATALEE primary report, Slamon et al 2024
- NATALEE prespecified five-year report, Crown et al 2025
- NATALEE six-year descriptive analysis, Slamon et al, Annals of Oncology journal pre-proof, 23 October 2026

Source reconciled; independent clinical, pharmacy and patient/PPI review pending